A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP

Gastroenterologist & Endoscopist

PN 421926PK

FOCUS GASTROENTEROLOGY

29 April 2026

Dr Vinith Menezes
The Neighbourhood Clinic

Dear Dr Menezes,

Re: Ms Jessica Edwards
DOB: 24/04/1980 Phone: 0415 669 281
Address: 119 Harold Street, THORNBURY, VIC, 3071
Email: jae.edwards4750@gmail.com
Medicare: 3467458966 / 1

Summary

- **Diverticulosis of the descending and sigmoid colon without evidence of active inflammation or structural change.**
- **Chronic left iliac fossa pain likely secondary to visceral hypersensitivity.**
- **Commencement of low-dose nortriptyline for neuromodulation.**
- **No action required by you.**

I had the pleasure of reviewing Jessica Edwards following her recent colonoscopy. You will be pleased to know that the procedure excluded any sinister pathology or significant structural changes within the bowel.

The colonoscopy identified diverticulosis within the descending and sigmoid colon. Crucially, there was no evidence of active diverticulitis at the time of the examination. We discussed that while Jessica has a history of CT-confirmed inflammatory flares, her persistent daily discomfort is likely driven by visceral hypersensitivity rather than ongoing inflammation. This is supported by the fact that she was experiencing symptoms at the time of the procedure despite a normal mucosal appearance.

We also explored the possibility of multifactorial pain, including gynaecological factors. Jessica mentioned that you have discussed the possibility of endometriosis with her, which is a plausible co-contributor to her symptoms. I have advised her that neuromodulators can be effective in managing the hypersensitivity associated with both diverticular disease and endometriosis.

Impression and Plan

I have initiated a trial of low-dose nortriptyline, starting at 10 mg at night. If she tolerates this without significant side effects such as somnolence or xerostomia, she has been advised to increase the dose to 20 mg after one week. I have reviewed her use of Intuniv for ADHD and do not anticipate any pharmacological interactions. I have encouraged a very gradual reintroduction of dietary fibre to help mitigate the risk of future diverticulitis, although she remains understandably cautious. For definitive acute flares, she will utilise a five-day course of antibiotics. I have scheduled a follow-up appointment in eight weeks to assess her clinical progress.

All appointments and enquires

Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

EDWARDS, JESSICA ANGHARAD : SVP 766640

DOB: 24/04/1980 Age: 46 years

119 Harold St, Thornbury 3071

Results from 11/04/2026 ..... Episode# 26P258062 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B009377 **CLINICAL NOTES** Random colon. **MACROSCOPIC DESCRIPTION** Specimen labelled "Random Colon". Three mucosal fragments measuring less than 1-3 mm. A1. Due to small nature of specimen, it may not survive processing. **MICROSCOPIC DESCRIPTION** The sections show colonic mucosa with maintained crypt architecture. There is a normal complement of chronic inflammatory cells in the lamina propria. The subepithelial collagen plate is of normal thickness. No active inflammation or granulomata are seen. There is no dysplasia or malignancy. **PATHOLOGIST'S OPINION** **Random colon biopsy: Normal colonic mucosa.** Pathologist: Dr Rohini Joshi

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